# Case study: Deb

Condition: **Cerebral palsy – dyskinetic (GMFCS II)**

58-year-old woman · Community occupational therapy home visit

Deborah (Deb) Fraser is a 58-year-old woman with **dyskinetic cerebral palsy** who lives in a lowset brick house in Toowoomba with her 86-year-old mother, Margaret, who has dementia. Deb is classified at GMFCS level II. She walks without an aid at home and has always had involuntary movements of her arms, neck and face. She retired early from her job as a library technician to care for her mother, and she drives a modified automatic car with a steering spinner knob. Deb is an NDIS participant, and Margaret receives Support at Home services through My Aged Care.

A letter from Deb's GP four months ago describes her cerebral palsy as "long-standing and stable" and notes that her recent blood tests were normal. Her NDIS plan records that she manages her own personal care and her mother's meals and medications independently.

You have been asked to complete a home visit to assess Deb's request for grab rails beside the toilet and in the shower. Deb meets you at the door, cheerful and organised. She has a brochure for a lift chair she would like for her mother and a written list of questions about what her NDIS plan will fund. Margaret sits in the lounge room, asking every few minutes when they are leaving. In the kitchen, her mother's medications are neatly sorted into a weekly pill organiser.

While Deb makes you a cup of tea, a mug slips from her right hand and breaks on the floor. She laughs. "That's the third this week. My hands have gone a bit useless," she says. She tells you her hands have felt "tingly and numb, like I'm wearing gloves" for about three months, and that her neck "aches all the time now, especially when I look up". She rubs the back of her neck while she talks.

In the bathroom, Deb demonstrates how she steps over the shower hob. She moves stiffly, catches her toe on the hob and grabs the towel rail to steady herself. She tells you she has fallen "four or five times" since winter and that her legs feel "stiff and heavy, like wading through mud". Most of the falls happened when she was hurrying to the toilet or turning quickly. When you ask how this compares with a year ago, she shrugs. "I've always been wobbly. It's just getting old with CP, isn't it?"

When you ask about the toilet, she lowers her voice. "I've had a few accidents lately. I can't always get there in time." She quickly adds, "Please don't make a fuss. Mum's much worse than me, and she's the one who needs looking after."

There is no clear documentation regarding whether Deb's GP knows about her hand symptoms, falls or bladder changes, which medications she takes, whether she has ever had imaging of her neck, or who could care for Margaret if Deb became unwell.

Deb glances at the clock and helps Margaret into her cardigan. "We'll have to finish up, love," she says. "Mum's memory clinic appointment is at eleven in town, and if we're late they'll cancel it." She picks up her car keys, fumbles them and drops them on the floor. "See? Butterfingers," she laughs, bending carefully to pick them up while holding onto the bench.

**You are the occupational therapy student completing a home visit with Deb and your clinical supervisor is not currently in the room.**

**Present Deb's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.**

# ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

## I Introduction

*≈ 20 s*

Who you are, your role, who the client is and why you need to talk now.

## S Situation

*≈ 40 s*

A brief but complete summary of what is happening and the immediate concern.

## B Background

*≈ 1 min*

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

## A Assessment

*≈ 2 min 20 s*

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

## R Recommendation

*≈ 2 min 40 s*

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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